CHAPTER

6

The Patient Who Doesn't Wear Their Aligners

"Well done is better than well said."

— BENJAMIN FRANKLIN

Sometimes, looking at the aligners in the patient's mouth is enough to see that things are not going as planned. But not every loss of tracking means the same thing, and, more importantly, not every case calls for the same clinical response.

If a single tooth is not tracking perfectly with the aligner, I do not automatically assume the patient is not following instructions, and I do not necessarily stop the sequence to take a new scan. We know that some movements are less predictable than others: a maxillary lateral incisor that needs a complex rotation, a difficult extrusion, or another particularly demanding movement may lag behind even when a patient wears their aligners impeccably. If that movement is not essential

to continuing treatment and its failure to occur will not interfere with subsequent movements, I often prefer to continue with the planned sequence and address it later during the refinement phase.

The situation changes when the loss of tracking no longer involves just one tooth but starts to become widespread. **That is why I normally schedule the first check during the sequence after about two months.** I consider that long enough to get a sense of how the patient is actually wearing their aligners and to begin seeing any loss of tracking, but short enough that, in most cases, I can intervene before the discrepancy becomes too significant. If I notice at that visit that several teeth are starting to lag behind their planned positions, we can often still restore tracking by changing the patient's behavior or temporarily slowing the sequence, without having to stop everything and take a new scan.

When I see at this first check that the loss of tracking no longer involves a single movement but is progressively affecting more teeth, I inevitably begin to suspect strongly that the patient is not wearing their aligners for enough hours. That is precisely when one of the most common communication problems in aligner treatment begins. To understand what is happening, I start with a very simple question:

"How are things going with your aligners? What time of day is it hardest for you to put them back in?"

That question helps me more than a general *"Are you able to wear them all the time?"*, to which the patient will almost inevitably say yes. So I try to get a clearer picture of what happens during a typical day, asking when they take the aligners out, how long meals last, what happens at work, and what happens when they go out in the evening or on weekends. I pay particular attention to the time between the end of a meal and when the aligners actually go back in, because it is often then, rather than during the meal itself, that the hours without aligners add up.

I do not approach this as an interrogation. I try to work with the patient to identify times when, without realizing it, they may be leaving their aligners out longer than they think. In the end, though, the patient may still be absolutely certain of their answer:

"No, Doctor. I really do wear them all the time."

At that point, I have two pieces of information that seem hard to reconcile: what I see clinically suggests that the teeth are not tracking with the aligners as they should, while the person in front of me insists with great conviction that they have followed every instruction to the letter.

The Behavior We Need to Change

The behavior we want to change is not simply *"following instructions more closely."* We need the patient to wear their aligners for the required number of hours, especially by reducing the times during the day when the aligners stay out of their mouth longer than they realize.

This matters because the problem very often does not come from a conscious decision to wear the aligners too little. It comes from the sum of many seemingly insignificant stretches of time: breakfast that takes longer than expected, coffee after lunch, a phone call before putting them back in, drinks before dinner, a dinner that runs long, or simply leaving them on the table and remembering them an hour later. Taken individually, these episodes seem negligible; added up over the course of a day, they can substantially reduce actual wear time.

That is why our first goal is not to determine whether the patient is telling the truth, but to turn a general *"I wear them all the time"* into a concrete enough account of their day to help us understand where the problem might lie.

The Mistake I Made: Trying to Convince the Patient

When faced with these situations, I chose the approach that seemed simplest and most logical to me. If I saw a widespread loss of tracking, I explained to the patient that they probably were not wearing their aligners enough and stressed the need to increase their wear time. I showed them what I was seeing clinically and said something like:

> *"Look, you can clearly see here that your teeth aren't tracking with the aligners. You need to try to wear them more."*

The problem was that I almost always got the same answer:

> *"But I do wear them."*

I could see the loss of tracking and kept pressing the point, while the patient insisted they had followed the instructions. The more I tried to convince them that the problem was their adherence, the more they felt the need to defend their behavior. After a few minutes, we were no longer discussing how to get treatment back on track, but which of us was right.

This dynamic can become even more pronounced when the patient in front of us is a very self-assured adult, perhaps someone with a prominent professional role who is used to making decisions, managing other people, and standing firmly by

their views. Telling that patient outright, *"That's not true. You aren't wearing your aligners,"* calls into question not only their adherence but also their credibility.

At that point, the clinical problem risks turning into a personal confrontation in which we both try, more or less consciously, to prove we are right. The paradox is that even if I won that argument, I would not necessarily have achieved what really matters to me. My goal should not be to convince the patient that they made a mistake in the preceding weeks, but to help them wear their aligners correctly from that point on.

It took me a while to understand this, and I probably needed one particular patient to change my perspective completely.

The Patient I Didn't Believe

I had treated Chantal's husband with aligners first, followed by their two children. All three had followed instructions closely, and their treatments had ended without any particular difficulties. So when she decided to straighten her teeth, too, I had no reason to expect anything different. Hers was, at least on the surface, a very simple case, with some crowding mainly around the lower incisors and nothing that, from a biomechanical standpoint, suggested any particular difficulty.

We started treatment, and at the first few checkups I realized that something was not going as planned: her incisors were moving much less than I expected, and the aligners were progressively losing tracking. She worked in fashion, traveled extensively, and had an especially busy professional life full of events, drinks before dinner, and other social occasions when it is easy to forget about aligners. For that reason, the explanation seemed almost obvious to me: she simply was not wearing them enough.

So I began stressing the importance of following the instructions, reminding her that she needed to wear the aligners for the prescribed number of hours and explaining that, if things continued this way, her teeth would not track as planned. But she kept telling me the same thing:

"Enzo, I do wear them."

I kept looking at those incisors, and I kept not believing her. This went on for more than six months, until Chantal told me she would be moving to London for work in a few months and asked me to finish treatment before she left. At that point, I thought I would eliminate once and for all what, in my mind, was still the main cause of the problem. I proposed putting brackets on her lower incisors, promising her that they would be perfectly aligned in a couple of months. I was fairly confident, because I

was convinced that once I eliminated adherence as a variable, those few teeth would align quickly.

Two months later, her incisors were still crooked.

That was probably the first time I seriously considered a possibility I had all but ruled out: perhaps this patient had never lied to me, and the problem I had attributed for months to her adherence might have been due, at least in part, to her biological response to orthodontic movement.

Teeth Don't Move at the Same Speed in Every Patient

That experience prompted me to look more closely at individual differences in the rate of orthodontic tooth movement, and I discovered that what I had observed clinically had a much stronger biological basis than I had imagined.

When we apply orthodontic force, we are not mechanically moving a tooth the way we might move an object. The force is the initial stimulus for a complex biological response involving the periodontal ligament, alveolar bone, osteoclasts, osteoblasts, inflammatory mediators, and numerous cellular and molecular mechanisms. The speed of this response varies considerably from person to person, and even when we try to standardize the force applied, different people can show very different rates of tooth movement.

One of the studies that best demonstrates this phenomenon was published by Giannopoulou and colleagues in the *European Journal of Orthodontics*. The authors used a standardized experimental protocol in which two premolars in each participant were moved buccally for eight weeks using a force of 1 N. Although the force was standardized and local or functional obstacles to movement had been ruled out, the amount of movement varied enormously between individuals, ranging from just 0.6 mm to 5.8 mm, with a mean of 2.7 mm. Even more interestingly, teeth belonging to the same individual tended to behave similarly, showing a strong within-person correlation. The authors concluded that this wide variability made it possible to identify *slow movers* and *fast movers* within the sample.

The concept has also been examined in subsequent clinical studies. In a randomized trial on extraction-space closure, for example, the authors explicitly used the terms *fast movers* and *slow movers* to distinguish patients who showed different amounts of tooth movement over the same period. In that particular study, cases with movement of at least 1 mm per month were defined as *fast movers*. Of course, that threshold is not a universal definition we can apply in clinical practice, and it does not allow us to simply sort all our patients into two categories. But it shows that

differences in individual rates of movement are real and that the concept is used in the orthodontic literature itself.

The reasons for this variability are probably numerous and are not yet fully understood. Orthodontic tooth movement depends on the tissues' biological response to force, and researchers have studied individual differences in the molecular systems involved in bone and periodontal ligament remodeling. Some studies have also investigated a possible genetic contribution, particularly polymorphisms in genes involved in the RANK, RANKL, and OPG systems and in other mediators of the orthodontic response. Interestingly, in this area too, the authors explicitly refer to the clinical phenomenon of *slow movers* and *fast movers*, suggesting that at least part of the difference in movement rates may depend on each person's biological characteristics.

This biological variability, of course, also existed when we treated patients with fixed appliances. In that setting, though, a *slow mover* was mainly a matter of time. If a tooth had not yet reached the desired position, in most cases we could simply leave the same archwire in place longer and wait for the biological response to run its course. As my great mentor Davide Mirabella has always told me: *"Let them cook."* Let them cook. Sometimes the best thing to do was simply give the teeth the time they needed to move.

The fact that a patient needed more time did not necessarily mean we had to change the treatment sequence: we could adjust the pace of treatment to the speed at which their teeth were actually moving.

With aligners, the problem takes on a different dimension. It is no longer just a question of how long a tooth takes to move, but of whether its biological rate of movement can stay in sync with the planned sequence. Each aligner is designed on the assumption that the movement planned for the previous aligner has occurred to a sufficient degree. If a patient is a *slow mover* and we keep switching aligners at a pace that is too fast for their biological response, a tooth can gradually fall behind its planned position. The next aligner then tries to fit a tooth that has not yet reached that position, and the discrepancy can build with each change until it becomes clinically apparent as a loss of tracking.

This is probably one reason why the concept of a *slow mover*, though far from new, becomes particularly relevant in aligner treatment. With fixed appliances, we could often simply afford to wait longer; with aligners, we need to match the pace at which we advance through the sequence to the pace at which that patient's biology can actually produce movement.

None of this means, of course, that a widespread loss of tracking should automatically be attributed to biology, or that being a *slow mover* is the most likely explanation whenever aligner treatment does not go as planned. Adherence remains one of the first variables to consider when several teeth begin losing tracking at the same time. But the literature on *slow movers* reminds us of something equally important: from clinical observation alone, we cannot always know for certain how much of what we are seeing is due to the patient's behavior and how much may be influenced by their individual biological response.

In the end, that was exactly what Chantal had taught me. In her case, I had eliminated adherence as a variable by switching to fixed appliances, and her teeth had continued to move slowly: **she really was a *slow mover*.** But that experience also helped me understand something else. If a slower biological response could genuinely explain a loss of tracking, then I could raise that same clinical possibility when I faced a patient whose adherence I strongly suspected was the problem instead, without accusing them or having to prove that what they were telling me differed from what they were actually doing.

Over the years, that insight gave rise to one of the strategies I use most often.

I Don't Have to Prove the Patient Is Lying

Today, when I see a patient with a widespread loss of tracking, I no longer start with an accusation or immediately try to convince them that they are not wearing their aligners. First, I try to work out with them what is happening by going through their daily habits and looking for times when the aligners stay out of their mouth longer than they realize. This step matters in light of everything we saw in the first part of the book about reconstructive memory and the gap that can exist between what we actually do and how we remember and describe our behavior.

If, after this conversation, the patient continues to assure me that they are wearing their aligners exactly as prescribed, I no longer try to prove otherwise. Instead, I deliberately accept their account and introduce another possible explanation:

> *"Look, we have two pieces of information we need to put together. I can see that your teeth aren't tracking with the aligners at the rate we expected, and you're telling me you're wearing them correctly. If that's the case, we need to consider another possibility."*

At this point, the patient usually pays close attention, because they do not feel accused and want to understand what the other explanation might be. I continue:

"Teeth don't move at the same speed in everyone. Most patients respond within the time frames we normally use, but there is a small percentage of people whose biological response is considerably slower. The literature even uses the terms fast movers and slow movers. If you're wearing your aligners as prescribed and your teeth keep falling behind, we need to consider the possibility that you're a slow mover."

At this point, I have not contradicted the patient or asked them to admit anything. On the contrary, I have accepted their account. But precisely because I have accepted it, I need to draw a consistent clinical conclusion:

"If your teeth move more slowly, one week might not be enough for them to complete the movement planned for each aligner. If we kept changing aligners every seven days anyway, the gap between your teeth's planned and actual positions could gradually grow, making the loss of tracking worse and worse."

The consequence I propose is simple:

"For that reason, at least for now, instead of changing your aligner every week, we should change it every three weeks. Then we'll check how your teeth are responding and, if we see that tracking is improving, we can gradually try to speed things up by switching, for example, to a change every two weeks."

Changing aligners every three weeks is a cautious clinical decision on my part, not an interval the literature has identified as necessary for all patients with a slower biological response. For the patient, though, the shift from seven to twenty-one days makes a consequence that may have seemed abstract immediately concrete: if we needed three times as long as planned for each aligner, and kept to that pace, the overall treatment could even take three times as long.

And this is often when the most interesting thing happens.

The patient thinks about it for a few seconds and spontaneously begins to describe their behavior a little differently:

"Well, Doc, now that I think about it, maybe it isn't quite twenty-two hours every single day. On weekends I sometimes wear them less, and when I go out, they might stay out longer. If I really make an effort to wear them all the time, can we keep changing them every week?"

It is important to understand that I have not asked them to confess anything. I have not said I do not believe them, and I have not tried to prove they were lying to me. I have simply taken what they told me seriously and shown them what the consistent clinical consequence would be if, despite wearing their aligners correctly, their teeth truly could not keep up with the planned pace.

Once the patient understands that this could mean going from seven to twenty-one days and therefore greatly prolonging treatment, they also start to see those hours without aligners differently. What, just a few minutes earlier, they had summed up as *"I wear them all the time"* can become *"Actually, a little less on weekends,"* or *"When I go out, they sometimes stay out quite a while."*

And when that happens, in most cases **I do not switch to three-week changes at all**. If the patient spontaneously acknowledges that they have worn their aligners for fewer hours than prescribed and tells me they want to try to be more consistent, I keep the weekly changes and give them a chance to show it through what happens over the following weeks. Three-week changes remain the consistent clinical consequence when the patient continues to assure me that they have worn their aligners correctly.

In other words, I do not use a slower pace as punishment. I use time as a motivator to make the consequence of what I am seeing concrete and allow the patient to choose whether to change their behavior.

At This Stage, Time Is the Most Powerful Motivator

As I have already described, Time, Outcome, and Investment are three of the motivators I use most often when communicating during aligner treatment, but they do not all carry the same weight at every stage of therapy.

When the patient started treatment only a few months ago, using the investment as a motivator generally makes little sense: treatment has just begun, and the patient has only recently paid for it. The outcome certainly remains important, but at this stage it can still feel relatively far away, and the patient may have trouble grasping how poor adherence today could jeopardize something they will not see for many months. Time, on the other hand, is immediately understandable.

If someone began treatment expecting to change aligners every week and to finish in roughly fifteen or eighteen months, hearing that they might now need to wear each aligner for three weeks makes the consequence extremely concrete. We are no longer talking vaguely about the possibility that *"treatment will take longer"*: the patient immediately sees that the pace of treatment could become three times slower than they had imagined at the start.

So I explain:

> *"We inevitably have to allow for the possibility that treatment could take much longer. If your teeth really need two or three times as long to keep up with each aligner, we can't expect treatment to finish in the time we originally planned. First*

we need to find out the pace at which your teeth can actually keep up with the aligners, and then we can give you a more reliable estimate."

I do not need to tell the patient that an eighteen-month treatment will definitely take four and a half years, because I cannot know that, and it would be wrong to turn a possible consequence into a certain prediction. But I can help the patient understand that, **if the pace really remained one aligner every three weeks instead of every seven days, the time required could be as much as three times longer**.

It is precisely the difference between a vague consequence and a concrete one that makes time as a motivator so effective. Saying *"if you don't wear them, treatment will take longer"* is something the patient has probably heard before, and it can remain entirely abstract. Going from seven to twenty-one days, on the other hand, translates that same consequence into something they can see, measure, and relate to their goal.

A Strategy That Works in Either Scenario

What interests me most about this approach is that I do not need to know with absolute certainty which of the two explanations is correct.

If the patient really is wearing their aligners twenty-two hours a day and has an unusually slow biological response, slowing down the changes and reassessing tracking is a cautious choice consistent with what I am seeing clinically.

If, instead, as is probably the case in most situations where I use this strategy, the patient is wearing their aligners for far fewer hours than they say, I have not accused them of lying or forced them to defend themselves. I have simply shown them a consequence of their current behavior that matters to them right now.

Very often, that is what prompts them to reconsider their account on their own. The patient does not have to jump abruptly from *"I wear them all the time"* to *"I was lying to you."* They can simply pause, think back over their week, and recognize that perhaps *all the time* did not really mean twenty-two hours every day. If they then decide to make a greater effort, I can keep the weekly changes and see whether their words are followed by a real change in behavior.

If, instead, they continue to insist that they are already wearing the aligners for the prescribed number of hours, I have no reason to argue: I accept their explanation, slow down the changes, and assess the response.

Either way, I have achieved what really mattered to me: **a concrete decision about what will happen from that point on.**

A Confession Doesn't Move Teeth

I think this has been one of the most important changes in how I communicate with patients who struggle with adherence. For a long time, I thought my job was to make the patient understand that they were not doing what I had asked, almost as though admitting the mistake were a necessary condition for getting back on track. Over time, though, I realized that getting that admission was not what I needed at all.

Paradoxically, today that kind of confession comes more often precisely because I have stopped seeking it. When the patient does not feel accused and instead understands the concrete consequence of what we are seeing, they can reconsider their behavior without feeling that changing their story means admitting defeat.

If they tell me, *"Actually, I wore them less over the weekend, but starting today I'm really going to make an effort,"* that information is certainly useful to me, because it allows me to keep the weekly changes and see whether better adherence is enough to restore tracking. But the confession is not what has therapeutic value: what matters is what the patient does next.

If, instead, after ten minutes of discussion, I managed to force them to admit, *"You're right, I was actually wearing them twelve hours a day,"* I might have the satisfaction of knowing I had interpreted the loss of tracking correctly, but clinically I still would not have achieved what really matters to me. **A confession does not move teeth; a change in behavior does.**

The *slow mover* strategy allows the patient to change their behavior without forcing them to admit that what they had told me just minutes earlier was not entirely true. It gives them a way to save face while making it much harder for them to ignore the consequence of continuing as they have been.

If we put someone in a position where they have to choose between admitting they have not followed our instructions and continuing to defend their account, they may choose the latter even when it does nothing to help them achieve the result they want. If, instead, we set that argument aside and focus on the clinical consequences and possible solutions, we allow them to change their behavior without first having to openly acknowledge that they made a mistake.

That is why, when the clinical evidence and the patient's account do not match, I now spend much less time trying to establish who is right and much more time finding a strategy that can work regardless of the explanation. **We do not need to prove to the patient that we are right; we need to create the conditions for them to start doing the right thing.**

PRACTICAL SCRIPT

When I observe a generalized loss of tracking, after reviewing the patient's habits with them and hearing them confirm that they are wearing their aligners correctly, I can frame the conversation this way:

> *"I can see that several teeth aren't keeping up with the aligners at the pace we expected. We've gone over how you're wearing them, and you've confirmed that you're wearing them correctly, so we need to consider another possibility. Teeth don't move at the same speed in everyone, and some patients have a slower biological response to orthodontic tooth movement. The literature also uses the terms fast movers and slow movers, and if you're wearing your aligners as prescribed, you may be in that second group.*
>
> *If your teeth move more slowly, one week may not be enough for them to reach the position each aligner is designed to achieve. If we kept changing aligners every seven days, that small lag could gradually grow, and tracking would get worse and worse. That's why, if you're wearing them for the prescribed number of hours, we need to give your teeth more time and switch, at least initially, from one week to three weeks per aligner.*
>
> *That would make the pace of treatment three times slower. I can't tell you today that the entire treatment will take three times as long, because we first need to see how your teeth respond, and we may be able to speed things up again. But if we had to maintain this pace throughout treatment, the total time could increase substantially. So now we need to find out the pace at which your teeth can actually keep up with the aligners."*

At this point, I give the patient room to respond. If they say:

> *"Doc, now that I think about it, maybe it isn't always twenty-two hours. If I really make an effort, can we try to keep the weekly changes?"*

my answer can be just as simple:

> *"Of course. If you think there are hours we can make up, I'd rather give you that chance. Let's keep the weekly changes, but from today on, we need to be very consistent. At your next visit, tracking will tell us whether that change was enough."*

If, instead, the patient continues to assure me that they are already wearing the aligners for the prescribed number of hours, I do not press the issue or try to get them to change their story: I slow down the changes as I explained and assess the response later.

Why This Protocol Works

- **Reducing Reactance**

Directly contradicting the patient and telling them they are not wearing their aligners as they claim may make them defend their position even more strongly. Initially accepting their account and proposing an alternative explanation avoids a confrontation and keeps the conversation focused on solving the problem.

- **Saving Face**

The *slow mover* strategy allows a patient who has not worn their aligners enough to change their behavior without being forced to admit they lied. They can simply reconsider what they meant by *"all the time"* and acknowledge that they missed some hours. That way, changing their behavior does not first require a personal defeat.

- **Making the Consequence Clear**

Saying *"you need to wear them more"* tells the patient what to do, but does not necessarily make clear why they should do it. Going from seven to twenty-one days, on the other hand, translates the consequence into something they can immediately understand.

- **Time as a Motivator**

In the early stages of treatment, the patient has only recently made the investment, and the final outcome may still feel far away. Time, meanwhile, is an immediate and easily understood consequence. The prospect of moving at one-third the pace can quickly change how the patient values those hours when the aligners are out.

- **Feedback**

The protocol does not end with the conversation. If the patient decides to keep the weekly changes and make a greater effort, tracking at the next visit provides the feedback we need to see whether the change in behavior was enough. If, instead, we slow down the changes and tracking still does not improve, we must also be willing to reconsider our clinical hypothesis.

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Adapting Communication to DISC Profiles

D

Dominance

Main motivators: time and control.

With a Dominance-style patient, I would avoid a direct confrontation, because openly questioning what they are telling us could quickly turn the conversation into a contest. It is more effective to present the two possibilities clearly and let what the patient does next determine which path we can take.

> *"If you're already wearing them for the right number of hours, we need to slow down because your teeth need more time. If, instead, you think you could be more consistent, we can keep the weekly changes and see right away whether tracking improves. We'll have our answer at your next visit."*

I

Influence

Main motivators: the outcome and motivation.

With an Influence-style patient, I would avoid an overly long or technical explanation and focus more on the possibility of getting things back on track quickly and keeping the original pace if they make a real change in their behavior.

> *"We can still try to keep the weekly changes. If you know you wore them less on some days, let's get back to the prescribed wear time starting today and see how your teeth respond. If tracking is back where we want it at your next visit, we'll continue this way."*

S

Steadiness

Main motivators: reassurance and a clear path forward.

With a Steadiness-style patient, it is important to keep them from interpreting the situation as a sign that treatment is failing. I would explain that we have two possible paths and that we can monitor either one over time.

> *"This doesn't mean the treatment isn't working. We just need to find out whether your teeth need more time or whether we can make your wear time more consistent. If you think you can be more consistent, we'll keep the weekly changes for now and look at how your teeth are responding together at your next visit."*

Adapting Communication to DISC Profiles

CONTINUED

C

Conscientiousness

Main motivators: data and explanation.

With a Conscientiousness-style patient, I would make greater use of what we can observe clinically, showing them how the aligners are tracking and explaining the two possibilities precisely, along with how the next visit will help us distinguish between them.

> *"Clinically, we can see that your teeth have fallen behind the planned positions. If you're already wearing the aligners for the right number of hours, we need to consider a slower biological response and give each aligner more time. If, instead, you think you've missed some hours of wear, we can keep the weekly changes for now, return to the prescribed wear time, and check at your next visit whether the gap has narrowed."*

CHAPTER 6

Protocol at a Glance

WHEN TO USE IT	At the first follow-up visit in the aligner sequence, usually after about two months, when a progressive, generalized loss of tracking appears. Also assess the treatment plan and biological response before attributing it to adherence.
GOAL	Restore tracking by changing the patient's behavior or the aligner change interval, then assessing the response.
WHAT TO DO	1. Review the patient's day: meals, work, evenings, weekends, and how long they wait before putting their aligners back in. 2. If the patient confirms that they are wearing them as prescribed, also consider a slower biological response. 3. Discuss moving from seven to twenty-one days per aligner, explaining that this would significantly slow treatment. Twenty-one days is a choice in the protocol described here, not a universal prescription. 4. If the patient acknowledges that they have not worn the aligners enough, agree on the prescribed hours of wear and keep the weekly changes when clinical conditions allow. 5. If the patient confirms that they are wearing the aligners as prescribed, slow down the changes and reassess tracking, then decide whether to maintain that pace or speed up. 6. If tracking does not improve, reconsider the initial clinical hypothesis as well.
WHAT TO CHECK	The distribution of the loss of tracking, hours and routine of wear, the aligner change interval, and the response at the next visit. A promise to make a greater effort must be followed by a verifiable change.